

How should I diagnose type 1 diabetes in an adult?

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- **Diagnose type 1 diabetes on clinical grounds in adults presenting with hyperglycaemia (random plasma glucose more than 11 mmol/L), bearing in mind that adults with type 1 diabetes typically (but do not always) have one or more of the following:**
 - Ketosis.
 - Rapid weight loss.
 - Age of onset younger than 50 years.
 - Body mass index (BMI) below 25 kg/m².
 - Personal and/or family history of autoimmune disease.
- **Do not discount a diagnosis of type 1 diabetes if an adult presents with a BMI of 25 kg/m² or above, or is aged 50 years or older.**
- **If type 1 diabetes is diagnosed, refer the person immediately (on the same day) to a diabetes specialist team to confirm the diagnosis and provide immediate care.**
- **Do not routinely measure C-peptide and/or diabetes-specific autoantibody titres to confirm the diagnosis of type 1 diabetes.**
 - C-peptide and/or diabetes-specific autoantibody titres may be measured (usually in secondary care) if:
 - Type 1 diabetes is suspected but the clinical presentation includes some atypical features (for example age 50 years or older, BMI of 25 kg/m² or above, or slow evolution of hyperglycaemia), *or*
 - Type 1 diabetes has been diagnosed and treatment started but there is a clinical suspicion that the person may have a monogenic form of diabetes, and one or both tests may guide the use of genetic testing, *or*
 - Classification of diabetes is uncertain and confirming type 1 diabetes would have implications for availability of treatment, for example continuous subcutaneous insulin infusion (CSII or 'insulin pump') therapy.

Basis for recommendation

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